

A FRAMEWORK IN CHAPTERS

Chapter Fifteen

Compulsory Existence

*Suicidism · the enforcement
of existence on those who would leave*

Compulsory Existence is the system that enforces existence on those who would rather not exist, criminalises the attempt to leave, and removes the option of a self-determined end. This chapter runs it through the five domains — from Section 327 of the Criminal Code to the prayer camp for the suicidal to the slow death of the person who is forced to live and who lives in agony.

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CHAPTER 15 OF AN ONGOING FRAMEWORK

Compulsory Existence

Compulsory Existence is the fourteenth Fundamental Matrix. It is the system that enforces existence on those who would rather not exist, criminalises the attempt to leave, and removes the option of a self-determined end. The framework also calls it Suicidism — the oppression of the suicidal subject, the treatment of the wish to leave as crime, sin, or sickness rather than as a sovereign decision about one's own existence.

Compulsory Existence overlaps with Retributivism (Chapter 13, which criminalises the suicide attempt under Section 327 of the Criminal Code, with up to one year's imprisonment) and with Sanism (Chapter 12, which treats the suicidal mind as mad). But the three matrices are not the same. Retributivism ranks by the retributive logic. Sanism ranks by cognitive capacity. Compulsory Existence ranks by the enforcement of existence itself — the demand that one must live regardless of whether one wants to. The framework separates them so that the compulsory-existence logic specifically can be studied and dismantled.

A note on method. The Interpersonal, Disciplinary, and Structural sections take the naive approach — each triad described as if happening for the first time, with no networks, residues, or Canon referenced. The Hegemonic Domain section is where Canon and Practice finally enter the picture.

READING NOTE — SENSITIVE CHAPTER

This chapter deals with suicide, forced existence, and the criminalisation of the wish to leave. The framework treats the subject analytically, not prescriptively. It does not encourage suicide. It does ask why the wish to leave is treated as crime, sin, or sickness rather than as a sovereign decision. If you are reading this chapter and are in distress, please contact a qualified mental health professional. The framework's point is not that existence is never worth sustaining — it is that the enforcement of existence is a matrix, and matrices must be studied.

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SECTION 1

1. Definition: What Compulsory Existence Extracts, Enforces, and Removes

Compulsory Existence is the system that enforces existence on those who would rather not exist. The theoretical foundations draw on Jasbir Puar's *The Right to Maim* (2017), which showed how the state claims the right not just to kill but to maim — to keep bodies alive in states of debility. Puar's analysis extends to Compulsory Existence: the state claims the right to force the subject to exist, even when existence is agony.

Achille Mbembe's *Necropolitics* (2003) showed that sovereignty is the power to dictate who may live and who must die. Compulsory Existence is the other side of necropolitics: the power to dictate that one must live, even when one would rather die. Giorgio Agamben's *Homo Sacer* (1998) supplied the concept of 'bare life' — life stripped of political meaning, kept alive as biological fact. The suicidal person under Compulsory Existence is reduced to bare life.

Lisa Guenther's *Solitary Confinement* (2013) showed how forced existence — the deliberate maintenance of life under conditions that make life unbearable — is itself a form of violence. Thomas Szasz's *The Myth of Mental Illness* (1961) and *Suicide Prohibition* (1986) argued that the medicalisation of suicide and the criminalisation of the suicide attempt are forms of state control over the subject's body, not forms of care.

For the historical context, Émile Durkheim's *Suicide* (1897) supplied the foundational sociological analysis, treating suicide as a social fact rather than an individual act. Jennifer Michael Hecht's *Stay* (2013) documented the history of anti-suicide arguments in Western thought. Judith Butler's *Frames of War* (2009) supplied the concept of 'grievable lives' — the question of which lives are treated as worth living and which as worth grieving. The WHO World Suicide Report (2014) supplied the global data.

1.1 What Compulsory Existence Extracts

Compulsory Existence extracts five things. First, the subject's **bodily autonomy**: the right to decide what happens to one's own body, including the right to end it. The suicidal person who is hospitalised, medicated, or chained loses the right to their own body. Second, the subject's **self-determination**: the right to decide whether to continue existing. The suicidal person who is forced to live loses the right to the most fundamental decision a being can make.

Third, the subject's **right to refuse**: the right to refuse treatment, medication, hospitalisation, and surveillance. Under Compulsory Existence, the suicidal person's refusal is treated as evidence of incapacity, which is then used to justify overriding the refusal. Fourth, the subject's **right to pain relief**: the right to adequate pain management at end of life, which is denied in Nigeria through the

lack of palliative care legislation and the opioid restriction.

Fifth, the subject's **right to a dignified end**: the right to die without pain, without surveillance, without coercion, and with the company of chosen people. Under Compulsory Existence, the only available deaths are the dangerous, the painful, the solitary, and the illegal. The safe, supervised, and dignified death is not available, because the system prohibits it.

1.2 What Compulsory Existence Enforces

Compulsory Existence enforces four standards. First, **existence as obligation**: the assumption that living is mandatory, that the subject owes existence to their family, their community, their God, or their state. The subject who does not want to exist is treated as violating an obligation.

Second, the **sanctity of life as non-negotiable**: the theological and philosophical principle that life is sacred and cannot be ended by choice. The principle is present in the colonial Christian Canon, in the medical ethics Canon, and in the legal Canon. Third, the **medicalisation of the wish to leave**: the treatment of the suicidal wish as a medical condition — a symptom of mental illness — rather than as a rational response to suffering or as a sovereign decision.

Fourth, the **surveillance of the suicidal**: the monitoring, tracking, and controlling of the person who has expressed the wish to leave. The surveillance is performed by the family, the hospital, the crisis line, the police, and the religious institution. Each standard is enforced by law, by medicine, by religion, and by the triads below.

1.3 What Options Compulsory Existence Removes

Concretely, Compulsory Existence removes the option of a self-determined end. The subject who has decided that existence is not worth continuing has no legal route to a safe, supervised, dignified death. The only available routes are the dangerous, the painful, and the solitary.

It removes the option of assisted dying — the practice, available in jurisdictions like Switzerland, the Netherlands, and Canada, of providing the subject with medical assistance to end their life under supervised conditions. Assisted dying is not legal in Nigeria. The subject who would prefer a supervised death to an unsupervised one has no option.

It removes the option of refusing treatment. The suicidal person who is hospitalised cannot refuse medication. The suicidal person who is chained in the prayer camp cannot refuse the exorcism. The suicidal person who is arrested under S327 cannot refuse the prosecution. It removes the option of pain relief at end of life — the lack of palliative care legislation in Nigeria means that the terminal patient who is in agony has no legal access to adequate pain management.

It removes the option of a dignified death — the right to die without pain, without surveillance, without coercion, and with the company of chosen people. Under Compulsory Existence, the only available deaths are the dangerous, the painful, the solitary, and the illegal. The safe, supervised, and dignified death is not available, because the system prohibits it. Each removal is enforced by the

Criminal Code, by the medical profession, by religion, and by the triads below.

SECTION 2 · NAIVE APPROACH

2. The Interpersonal Domain

The Interpersonal Domain contains four enforcement triads and four extraction triads. Each is described here as if happening for the first time — no networks, no residues, no Canon.

3.1 Enforcement Triad: Existence as Obligation Standard

The enforcer is the person who demands the subject continue existing. The family member who says ‘think of your children.’ The friend who says ‘you have so much to live for.’ The state that criminalises the attempt to leave. The religious leader who says ‘suicide is a sin.’ The enforcement is performed by guilt, by law, by theology, or by social pressure.

The subject conforms by continuing to exist. The conforming act is the continued existence itself — the subject wakes up each day, performs life, and does not act on the wish to leave. The conformity is invisible because it is performed as ‘normal life.’

The options the subject loses are concrete. The subject who is forced to exist loses the option to self-determination, which is to say, loses the option to the most fundamental decision a being can make. The subject who is guilted into existence loses the option to bodily autonomy. The subject who is criminalised for the attempt loses the option to liberty (overlapping with Retributivism, Chapter 13).

The witness is the person who watches the enforcement and says nothing, or who themselves enforce existence on their own loved ones. The Non-Extracting Beneficiary is the institution that benefits from the subject’s continued existence — the employer who keeps the worker, the family that keeps the member — without personally commanding the enforcement.

3.2 Enforcement Triad: Sanctity of Life as Non-Negotiable Standard

The enforcer is the person or institution that invokes the sanctity of life to prohibit the exit. The religious leader who says ‘life is sacred.’ The medical professional who says ‘my duty is to preserve life.’ The legislator who says ‘the law protects life.’ The enforcement is performed by theology, by medical ethics, by law, or by social convention.

The subject conforms by accepting the sanctity principle. The conforming act is the acceptance itself — the subject does not question the principle, because the principle is so deep that the alternative is invisible. The subject who would question the principle is treated as immoral, irrational, or mentally ill.

The options the subject loses are concrete. The subject who accepts the sanctity principle loses the option to a self-determined end, which is to say, loses the option to the most fundamental form of self-determination. The subject who is prevented from acting on their own assessment loses the option to sovereignty over their own existence.

The witness is the person who watches the sanctity enforcement and says nothing, or who themselves invoke the sanctity principle. The Non-Extracting Beneficiary is the institution that benefits from the sanctity Canon — the hospital that keeps the patient, the prison that keeps the inmate — without personally commanding the enforcement.

3.3 Enforcement Triad: Medicalisation of the Wish to Leave Standard

The enforcer is the medical professional who treats the suicidal wish as a medical condition. The psychiatrist who diagnoses the wish as a symptom of depression. The therapist who treats the wish as a cognitive distortion. The general practitioner who prescribes medication to alter the wish. The enforcement is performed by the diagnosis, by the prescription, by the hospitalisation, or by the treatment plan.

The subject conforms by accepting the diagnosis. The conforming act is the acceptance itself — the subject internalises the framing that their wish to leave is a symptom rather than a decision. The subject who refuses the diagnosis is treated as lacking insight, which is then used to justify overriding the refusal.

The options the subject loses are concrete. The subject who is diagnosed loses the option to their own framing of their experience, which is to say, loses the option to self-knowledge. The subject who is medicated loses the option to their own mind, which is to say, loses the option to the cognitive state that produced the wish. The subject who is hospitalised loses the option to liberty.

The witness is the person who watches the medicalisation and says nothing, or who themselves recommend the medical route. The Non-Extracting Beneficiary is the pharmaceutical industry that profits from the medication without personally commanding the medicalisation.

3.4 Enforcement Triad: Surveillance of the Suicidal Standard

The enforcer is the person or institution that monitors the suicidal subject. The hospital that places the patient on suicide watch. The family that removes sharp objects. The crisis line that reports the caller to the police. The police who conduct the wellness check. The enforcement is performed by the observation, by the environmental control, by the reporting, or by the police intervention.

The subject conforms by being observed. The conforming act is the submission to surveillance — the subject allows the observation, accepts the removal of objects, tolerates the reporting. The subject who resists is treated as non-compliant, which is then used to justify more surveillance.

The options the subject loses are concrete. The subject who is surveilled loses the option to privacy, which is to say, loses the option to the mental space required for self-determination. The subject who is reported to the police loses the option to safety, because the police introduction brings the threat of state violence. The subject who is watched loses the option to dignity.

The witness is the person who watches the surveillance and says nothing, or who themselves surveil their own loved ones. The Non-Extracting Beneficiary is the institution that benefits from the

surveillance data without personally commanding the surveillance.

3.5 Extraction Triad: Bodily Autonomy

The extractor is the institution that requires the subject's body be maintained. The hospital that prevents the exit. The prison that prevents the exit. The family that prevents the exit. The state that criminalises the attempt. The extraction is performed by the hospitalisation, by the medication, by the surveillance, or by the criminal prosecution.

The subject conforms by remaining in the body. What is extracted is the bodily autonomy itself — the right to decide what happens to one's own body, including the right to end it. The extraction transfers the autonomy from the subject to the institution, with the subject reduced to a body that must be kept alive.

The options the subject loses are concrete. The subject who is hospitalised loses the option to bodily movement, which is to say, loses the option to leave. The subject who is medicated loses the option to their own brain chemistry, which is to say, loses the option to the mind that wanted to leave. The subject who is chained in the prayer camp loses the option to bodily liberty.

The witness is the person who knows of the extraction and does not intervene. The Non-Extracting Beneficiary is the institution that benefits from the subject's continued existence — the hospital that bills for the bed, the pharmaceutical company that bills for the medication — without personally commanding the extraction.

3.6 Extraction Triad: Self-Determination

The extractor is the system that requires the subject's self-determination be forfeit. The psychiatric system that overrides the refusal. The legal system that criminalises the attempt. The religious system that condemns the wish. The family system that guilt the subject into continuing. The extraction is performed by the override, by the prosecution, by the condemnation, or by the guilt.

The subject conforms by surrendering the decision. What is extracted is the self-determination itself — the right to decide whether to continue existing. The extraction is the most fundamental extraction the framework studies, because it extracts the right to the most fundamental decision.

The options the subject loses are concrete. The subject who surrenders the decision loses the option to self-determination, which is to say, loses the option to be a sovereign being. The subject whose refusal is overridden loses the option to have their judgment respected. The subject whose attempt is criminalised loses the option to liberty.

The witness is the person who knows of the extraction and does not intervene. The Non-Extracting Beneficiary is the system that benefits from the subject's continued existence without personally commanding the forfeiture.

3.7 Extraction Triad: Right to Refuse Treatment

The extractor is the medical system that requires the subject accept treatment. The psychiatrist who prescribes involuntarily. The hospital that medicates involuntarily. The court that commits involuntarily. The prayer camp that chains involuntarily. The extraction is performed by the prescription, by the medication, by the commitment, or by the chaining.

The subject conforms by accepting the treatment. What is extracted is the right to refuse — the right to say no to the intervention that alters the mind or the body. The extraction is circular: the subject's refusal is treated as evidence of incapacity, which is then used to override the refusal.

The options the subject loses are concrete. The subject who is medicated involuntarily loses the option to their own brain chemistry. The subject who is hospitalised involuntarily loses the option to liberty. The subject who is chained in the prayer camp loses the option to bodily safety.

The witness is the person who knows of the extraction and does not intervene. The Non-Extracting Beneficiary is the pharmaceutical company that profits from the involuntary medication without personally commanding the extraction.

3.8 Extraction Triad: Right to a Dignified End

The extractor is the system that requires the subject's death be dangerous, painful, solitary, or illegal. The criminal code that prohibits the safe route. The medical system that prohibits the supervised route. The religious system that prohibits the dignified route. The extraction is performed by the prohibition, by the criminalisation, by the medicalisation, or by the theological condemnation.

The subject conforms by accepting the dangerous route or by not acting on the wish. What is extracted is the right to a dignified end — the right to die without pain, without surveillance, without coercion, and with the company of chosen people. The extraction transfers the dignity from the subject to the system that prohibits the dignified route.

The options the subject loses are concrete. The subject who is denied the safe route loses the option to a painless death. The subject who is denied the supervised route loses the option to a safe death. The subject who is denied the legal route loses the option to a death that does not criminalise the attempt or shame the family. The loss is absolute: the subject who is denied all routes is left with the dangerous, the painful, or the continuation of existence in agony.

The witness is the person who knows of the extraction and does not intervene. The Non-Extracting Beneficiary is the system that benefits from the subject's continued existence without personally commanding the prohibition.

Nascent forms of this system include: 'Suicide is a sin', 'Suicide is illegal', 'Suicide is selfish', 'Think of your family', 'Think of your children', 'It gets better', 'You have so much to live for', 'Permanent solution to a temporary problem', 'Call the hotline', 'How could you do this to us?', The forced hospitalisation, The forced medication, The surveillance of the suicidal patient, The removal of sharp objects, The psychiatric hold, The criminal arrest under S327, The prayer camp for the suicidal, The exorcism of the 'spirit of death', The crisis line as surveillance, The wellness check as police

intervention, The 'committed suicide' language, The post-suicide family shaming, The religious condemnation of the deceased, The erasure of the suicide from the family narrative.

SECTION 3 · NAIVE APPROACH

3. The Disciplinary Domain

When the eight triads above link into networks, Compulsory Existence scales. Both hierarchical and community-based forms are visible in every Nigerian institution that deals with the suicidal subject.

4.1 Disciplinary Hierarchy — Nested Stars

Peak Triad-Node

At the peak is the triad where the highest authority enforces existence on the layer below. In the medical hierarchy, the peak is the Chief Medical Director enforcing on consultants, who enforce on psychiatrists, who enforce on nurses, who enforce on the suicidal patient at the bottom. In the legal hierarchy, the peak is the Chief Judge enforcing on the High Court, which enforces on the Magistrate, which enforces on the police, who enforce on the suicidal person arrested under S327.

In the religious hierarchy, the peak is the General Overseer enforcing on the prophet, who enforces on the prayer camp inmate, who is the suicidal person chained for deliverance. In the family hierarchy, the peak is the parent who forces the child to live, who enforces on the child who must obey.

Middle Triad-Nodes

The middle nodes are where the chain's character is clearest. The psychiatrist who defers to the CMD in the morning enforces on the patient in the afternoon. The police officer who defers to the Magistrate in the morning arrests the suicidal person in the afternoon. The prophet who defers to the General Overseer in the morning chains the suicidal person in the afternoon.

Bottom Triad-Node

At the bottom is the suicidal person — the person who has expressed the wish to leave and who is now subject to every hierarchy simultaneously. The hospital that medicates them. The police that arrest them. The church that chains them. The family that guilts them. The bottom is where the system's violence is most concentrated, because the suicidal person is the one subject who has tried to exercise the most fundamental form of self-determination and who has been prevented by every institution.

4.2 Disciplinary Community — Distributed Network

A Disciplinary Community is a distributed network. The witness of one Compulsory Existence triad becomes the enforcer of another. Compulsory Existence spreads laterally through any person who has witnessed the enforcement of existence without dissenting.

The Suicide Prevention Industry

The suicide prevention industry circulates Compulsory Existence laterally across the public. The crisis line that may report the caller. The awareness campaign that frames the suicidal as needing rescue.

The training programme that teaches the public to identify and report. The industry reproduces the surveillance triad across the public, with no central command issuing the demand.

The Religious Anti-Suicide Networks

The religious anti-suicide networks circulate Compulsory Existence laterally across the religious community. The sermon that condemns suicide as sin. The prayer meeting that prays for the suicidal. The deliverance session that chains the suicidal. The networks reproduce the sanctity-of-life triad across the religious community.

The Family Surveillance Networks

The family surveillance networks circulate Compulsory Existence laterally across the family. The parent who checks the child's room. The sibling who monitors the sibling's mood. The spouse who removes the means. The networks reproduce the surveillance triad across the household.

The Social Media 'Reach Out' Culture

The social media 'reach out' culture circulates Compulsory Existence laterally across the digital public. The post that says 'if you're struggling, reach out.' The message that says 'I'm here for you.' The culture reproduces the existence-as-obligation triad across the digital public, with the 'reach out' framing placing the burden on the suicidal person to ask for help that may come with surveillance.

The Wellness Check Networks

The wellness check networks circulate Compulsory Existence laterally across the state. The crisis line that dispatches the police. The police who arrive at the door. The intervention that may escalate. The networks reproduce the surveillance triad across the state, with the wellness check as the point where the caring gesture becomes the police action.

SECTION 4 · NAIVE APPROACH

4. The Structural Domain

Run the Ship of Theseus test. When the original colonial legislators and original psychiatrists are dead and replaced, when the networks are disbanded and reform, what is consistent? What outlives both individuals and groups? What forces new Nigerians to re-form into the same patterns — not because anyone instructs them to, but because the residue makes any other pattern costly or invisible?

Compulsory Existence leaves all three residues. The criminal code, the medical system, and the religious Canon all carry Bureaucracy, Stratification, and Classification from Compulsory Existence.

5.1 Bureaucracy — Enduring Rules and Procedures

Compulsory Existence's bureaucracy is the codified enforcement of existence. **Section 327 of the Criminal Code Act 1916** criminalises attempted suicide with up to one year's imprisonment. **Section 326** criminalises aiding suicide with up to fourteen years. The Lunacy Act 1916, though replaced by the Mental Health Act 2021, established the framework of involuntary commitment that persists.

The psychiatric hold provisions allow the hospital to detain the suicidal person for observation. The involuntary commitment procedures allow the court to commit the suicidal person to a psychiatric facility. The lack of palliative care legislation means that the terminal patient in agony has no legal access to adequate pain management. The lack of assisted dying legislation means that the subject who has decided to leave has no legal route to a supervised death.

The WHO suicide prevention framework, as adopted by Nigeria, frames suicide as a public health problem to be prevented, not as a sovereign decision to be respected. The National Mental Health Policy 2013 and the suicide prevention strategy documents reproduce the medicalisation Canon. The original 1916 authors are dead. The bureaucracy lives.

5.2 Stratification — Enduring Zero-Sum Rankings

Compulsory Existence's stratification is the daily experience of existence-rank in every institutional encounter. **Living / wanting-to-die** — the person who wants to live ranks above the person who wants to die in moral standing, in social recognition, and in legal protection. **Compliant / non-compliant patient** — the patient who accepts treatment ranks above the patient who refuses.

Treatable / untreatable — the patient whose suicidality responds to treatment ranks above the patient whose suicidality does not. **Sane enough to refuse / not sane enough to refuse** — the patient who is judged to have capacity can refuse treatment; the patient who is judged to lack capacity cannot. The judgment is made by the psychiatrist, whose own interest is in treatment rather than refusal.

5.3 Classification — Enduring Categorical Divisions

Compulsory Existence's classification is the set of categorical names that organise the suicidal subject. **Suicidal / non-suicidal** — the universal pair, with non-suicidal as the default and suicidal as the deviation. **Compliant / non-compliant** — the medical pair, with compliant as the good patient and non-compliant as the problem.

Mentally ill / mentally well — the psychiatric pair, with mentally well as the default and mentally ill as the diagnosis that justifies the override. **Criminal / patient** — the legal pair, with the suicide attempt treated as both crime (S327) and symptom. **Sinner / saved** — the religious pair, with the suicidal as sinner and the prevented as saved. **Selfish / normal** — the moral pair, with the suicidal as selfish and the non-suicidal as normal. The classification does the work before any individual enforcer shows up.

SECTION 5

5. The Hegemonic Domain: The Ascension Ritual

Is Compulsory Existence ascended? Yes on all three counts. It produces all three structural residues including Stratification (so Internalised Domination applies). It has working Canon and Practice mechanisms. The vast majority of Nigerians cannot imagine a society where suicide is not criminalised, where the suicide attempt is not treated as crime or sickness, where the subject has the right to a self-determined end — despite the fact that the criminalisation is a colonial-era import from English common law.

6.1 Canon — Rewriting History

Canon is the present belief that the enforcement of existence has always been so. The origination of the structural residues — S327 of the Criminal Code, the psychiatric hold, the involuntary commitment — has been forgotten. The average Nigerian now takes the criminalisation of suicide as natural, eternal, and without alternative.

What is forgotten? The criminalisation of suicide is a colonial-era import from English common law. In pre-colonial English law, suicide was a crime — the property of the deceased was forfeit to the Crown, and the body was buried at a crossroads with a stake through the heart. The Suicide Act 1961 decriminalised suicide in England. But the Nigerian Criminal Code, drafted in 1916, retained the criminalisation, and Nigeria has not followed England in decriminalising.

Also forgotten: that pre-colonial Nigerian societies had more complex relationships with self-death. The Yoruba concept of the person who ‘went home’ (the person who returned to the ancestors). The Igbo practice of the elder who ‘went to join the ancestors’ when they deemed their time complete. These were not celebrated, but they were not criminalised. The colonial encounter imposed the English criminalisation, and the Canon now presents it as natural.

Also forgotten: that the ‘sanctity of life’ theology is a colonial Christian import. Pre-colonial Nigerian religions had more nuanced relationships with death, including the recognition that some deaths were the person’s own decision. The medicalisation of suicide is a 20th-century Western development, accelerating after the publication of Durkheim’s *Suicide* in 1897. The suicide prevention industry is a recent economic formation, accelerating from the 2000s. Each is treated as natural by the Canon, but each is a recent imposition.

Internalised Domination

Canon produces Internalised Domination because Compulsory Existence produces Stratification. The psychiatrist’s entitlement to the suicidal patient’s body is now natural, automatic, unquestionable. The psychiatrist who overrides the refusal does not experience this as a demand. They experience it as the

natural order of medical care. The police officer's entitlement to the suicidal person's liberty is natural. The family's entitlement to the suicidal person's continued existence is natural. The religious leader's entitlement to the suicidal person's soul is natural.

Internalised Authority

Canon produces Internalised Authority in the Compulsory Existence enforcer regardless of personal material gain. The right to force existence. The right to hospitalise. The right to medicate. The right to chain in the prayer camp. The right to arrest under S327. The right to surveil. The right to remove sharp objects. The right to conduct the wellness check. Each is performed by ordinary Nigerians daily — the psychiatrist who commits the patient, the police officer who arrests the attempt survivor, the prophet who chains the suicidal, the parent who removes the belt. The right to enforce feels natural, automatic, and legitimate.

Internalised Oppression

Canon produces Internalised Oppression in the suicidal subject and the family. The suicidal person who believes they are selfish. The suicidal person who believes they are sinning. The suicidal person who believes they are mentally ill. The suicidal person who believes they must live for others. The family who believes they failed. The teaching is silent; it is in the air.

The deepest form of this is the subject who has absorbed Compulsory Existence so thoroughly that they enforce it on themselves. The suicidal person who says 'I am selfish for wanting to leave' to explain why they do not act. The suicidal person who says 'I am sinning' to explain why they seek the prayer camp rather than the exit. The suicidal person who says 'I am mentally ill' to explain why they accept the medication that alters their mind. The family who says 'we failed' to explain why they did not prevent the attempt. The rebellion-capable subject has become the self-policing subject.

6.2 Practice — Rewriting Reality

Practice is the second mechanism of the Ascension Ritual. The interpersonal triad is repeated, again and again, until it becomes habit. The enforcer forgets they can choose not to enforce. The suicidal subject forgets they can rebel. The witness forgets they can dissent. Pierre Bourdieu called this *habitus* — the sediment of repeated practice in the body.

Lisa Guenther, in *Solitary Confinement* (2013), showed how forced existence — the deliberate maintenance of life under conditions that make life unbearable — is itself a form of violence that destroys the subject's capacity for selfhood. Jasbir Puar, in *The Right to Maim* (2017), showed how the state claims the right to keep bodies alive in states of debility. The body learns. The mind stops questioning. By adulthood, the Nigerian has performed the existence-as-obligation, sanctity-of-life, medicalisation, and surveillance triads so many times that the alternative — the right to a self-determined end — feels like a kind of amputation. The body has been trained. Practice has rewritten reality.

Death — Fast and Slow

Practice produces Death. Fast death in Compulsory Existence is spectacular: the suicide that is completed because the subject could not access a safe, supervised, dignified route — the subject who takes the dangerous route because the safe route is prohibited, and who dies alone, in pain, and without dignity. The death in the prayer camp from the chains and the exorcism — the suicidal person who is sent for deliverance and who dies from the conditions of the deliverance (overlapping with Sanism, Chapter 12, and Religious Dominance, Chapter 6).

The death from the police intervention in the wellness check — the suicidal person who is killed by the police who were called to ‘save’ them. This is not a hypothetical: there are documented cases across the world of wellness checks escalating to police killings. The death from the forced medication — the adverse reaction, the overdose, the interaction. The death from the psychiatric hold conditions — the overcrowded ward, the inadequate medical care, the neglect.

Slow death in Compulsory Existence is quiet. Chronic stress of being forced to exist — the suicidal person who has been prevented from leaving and who must now continue to exist in the agony that produced the wish. Mental health collapse from the involuntary commitment — the subject who is hospitalised against their will and whose mental health deteriorates further under the conditions of the hospitalisation. The slow death of the suicidal person who is forced to live and who lives in agony — the body that continues while the mind that wanted to leave is suppressed by medication.

The cumulative damage of the forced medication — the weight gain, the emotional numbing, the cognitive impairment, the sexual dysfunction, the metabolic syndrome. The cumulative damage of the surveillance — the loss of trust, the loss of privacy, the loss of the mental space required for self-determination. The family’s chronic guilt — the family who prevented the attempt and who now must watch the subject continue in agony, wondering whether they did the right thing. The erosion of trust between the suicidal person and their support network — the friend who reported to the police, the family member who removed the means, the therapist who broke confidentiality. None of these make the news. They are Compulsory Existence’s quiet tax.

Ruth Wilson Gilmore’s definition applies: Compulsory Existence produces group-differentiated vulnerability to premature death. The groups are the suicidal, the involuntarily committed, the chained in the prayer camp, the arrested under S327, the terminal patient denied pain relief, the family of the suicidal. The premature death is fast (the dangerous suicide, the prayer camp death, the wellness check killing, the forced medication death, the psychiatric hold death) or slow (forced existence stress, involuntary commitment collapse, medication damage, surveillance damage, family guilt, trust erosion). Every ascended matrix kills. Compulsory Existence kills those whose wish to leave does not fit the compulsory-existence model.

SECTION 6

6. When Did Compulsory Existence Rewrite History?

The framework's historical question: when did Compulsory Existence rewrite history in popular Nigerian living memory, such that the average Nigerian cannot recall a time when its structural residue was not present?

Pre-colonial Nigerian societies had more complex relationships with self-death. The Yoruba concept of the person who 'went home.' The Igbo practice of the elder who 'went to join the ancestors.' These were not celebrated, but they were not criminalised. The relationship was complex — some self-deaths were treated as shameful, others as the person's sovereign decision. The colonial encounter imposed the English criminalisation, which treated all self-death as crime.

Colonial codification imposed the enforcement of existence. The Criminal Code Act 1916, Section 327, criminalised attempted suicide. Section 326 criminalised aiding suicide. The colonial psychiatric system, established through the Lunacy Act 1916, medicalised the suicidal wish. The colonial church condemned the suicidal as sinful. Each was a political decision, not a natural development.

Post-independence replication continued the enforcement. The Nigerian Criminal Code retained S327 and S326. The Nigerian psychiatric system retained the involuntary commitment framework. The Nigerian church retained the theological condemnation. The Mental Health Act 2021, though a significant improvement on the Lunacy Act, did not decriminalise attempted suicide. England, the country that gave Nigeria the criminalisation, decriminalised suicide in 1961. Nigeria has not followed.

The tipping point in popular living memory: **circa 1916 for the Criminal Code** that criminalised attempted suicide; **circa 1960 for the consolidation of the medical model**. For Nigerians born after 1960, Compulsory Existence has 'always been' in its current form. They cannot recall a time when suicide was not criminalised, when the attempt was not treated as crime or sickness, when the subject had the right to a self-determined end.

The pre-compulsory-existence generation (born ~1860s-1880s) was the last to have living memory of pre-colonial relationships with self-death. By the 1916 Criminal Code, that generation had been displaced from policy-making, and the criminalisation had become the natural order. The question 'is the criminalisation of suicide natural?' has never been asked in popular Nigerian memory, because the criminalisation has been in place since before living memory began.

SECTION 7

7. Fill in the Blank: A Self-Examination

The framework asks two questions of every subject of every ascended matrix. The blanks are for you. They are not rhetorical. This chapter's blanks may be the most difficult in the framework, because they ask about the most fundamental form of self-determination: the right to decide whether to exist.

Question 1: When did Compulsory Existence rewrite your reality?

When did the system teach you that you could not rebel — that you must exist regardless? The moment was: _____. The year was: _____. The person or institution who first made you feel that challenging the enforcement of existence was impossible was: _____. The place was: _____. What you wanted to say but did not was: _____. What you did instead was: _____. What you told yourself about why you did not say it was: _____. The enforcement form you encountered was: _____. What you lost, in that moment, that you have not got back since, was: _____.

Question 2: How is Compulsory Existence causing your Death?

How is the system shortening your life right now? The form of Death I am most afraid of from Compulsory Existence is (fast/slow): _____. The specific way it is happening in my body is: _____. (For example: the chronic stress of being forced to exist; the mental health collapse from the involuntary commitment; the slow death of being forced to live in agony; the cumulative damage of the forced medication; the cumulative damage of the surveillance; the family guilt; the erosion of trust between me and my support network; the criminal record from the S327 arrest; the chains in the prayer camp; the loss of the right to a dignified end.) The matrix's name for what is happening to me is: _____. My own name for it is: _____.

If you filled in the blanks honestly, you have just located yourself inside Compulsory Existence. The next time the triad assembles — the next time the hotline is called, the next time the wellness check is dispatched, the next time S327 is invoked, the next time the prayer camp is recommended — you will be able to see the system moving through you. That sight is the beginning of abolition. And if you are now the enforcer in the triad — the psychiatrist, the police officer, the prophet, the parent — the sight is also the beginning of your refusal to enforce.

IF YOU ARE IN DISTRESS

This chapter has discussed suicide, forced existence, and the criminalisation of the wish to leave. If you are reading this chapter and are in distress, please contact a qualified mental health professional. The framework's point is not that existence is never worth sustaining. It is that the enforcement of existence is a matrix, and matrices must be studied. The framework asks for the right to study, not for the right to prescribe.

SECTION 8

8. Endnotes & References

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Coming Next

Chapter 16 will take up Meritism / The Significant — the system in which demonstrated capacity to capture collective attention and fascination grants social licence and exemption from ordinary rules. The framework's outline names it as the system that gives celebrities, influencers, and the famous a pass that ordinary people do not get. Meritism overlaps with Productivism (Chapter 8, which enforces productivity) and with Coloniality (Chapter 11, which ranks by colonial standards). The framework separates them so that the attention-capture logic specifically can be studied and dismantled.

If you are reading this chapter as part of the framework in progress, the order continues: Chapter 17 is Ethnic Majoritarianism, Chapter 18 is Territorialism, Chapter 19 is State Sovereignty / Statism, and so on through the Fundamental Matrices. Each chapter follows the same template established in Chapters 2 through 15. The pattern is now set.